

**إسم المريض رابعياً : مديحه محمد عبدالغنى البقراوى**

**المحافظة : القاهرة**      **الحى / المدينة : المعادى**

**العنوان : الاتحاد**

رقم البطاقة : 25102080100304      رقم التليفون : 01117054213

**تاريخ تسجيل الإستمارة : 21-09-2026**

**العمر(تاريخ الميلاد) : 08-02-1951 المهنة : ربه منزل**

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-  
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة  
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه  
التوقيع: (-----)

## التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

## نتائج الأبحاث : (مرفق)

### التشخيص النهائي : سرطان بالثدي (مبادرة)

**خطة العلاج :** علاج مبادرة سرطان الثدي علاج تدعيمى وتلطيفى [6579] **قسم العلاج :** الاورام

**التعليق على التشخيص : -----**

GR1-E  
جورستان  
روستاییلدانی  
لیدی حساب  
جورستان  
صحة الامانات المرفعة  
اللجنة و مدير

**الاسم: د/محمد کیلانی**

**الاسم: د/ندی حسن**

**الاسم: د/نرمين رمضان**

**الوظيفة: ----- التوقيع:**

**الوظيفة: ----- التوقيع:**

**الوظيفة: ----- التوقيع:**

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس**

**وتلتزم جهة العلاج بلائحة المجالس الطبية.**

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.**

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

## قرار اللجنة

### التشخيص النهائي :

### خطة العلاج :

### المستشفى :

### جهة العلاج : --

### ملاحظات : ---

**قسم :**

## نَفَقَات :

## ختم

## شعار الجمهورية

## مدير عام

## مديرية الشؤون الصحية

## رئیس

## المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                                 |                                                                                     |
|---------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> مديحه محمد عبدالغنى البقراوى<br><b>National ID:</b> 25102080100304 |  |
|---------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                                      |
|--------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment |
|--------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



Dms

فيما



٢٠٢٥/٠٧ ٢٥١٠٢٠٨٠١٠٠٣٠٤

مركز تربية والتعليم سابقا

ارملة

مسلعة

انثى



البطاقة سارية حتى ٢٠٢٢/٠٧/١٢

١





Presidential Program for Women's Health  
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة  
وزارة الصحة والسكان



4-Patient ID:

1- Name : مديحة محمد عبد القادر الشراوي

Sex

### MDT form

3- Medical number:  
Female

83255111

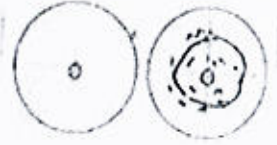
Old Med :

FN- MED

166-2025

2-ID number:

Patient Age : 74 Y



Family history

Family history  
+ve

Phone

Phone

01117054213

BMI

BM

REASON OF MDT

REASON OF MDT  
FIB

marital status

Marital status  
Married

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity  
diabetic, GII  
nephropathy  
bil

Menopausal  
status

Menopausal status  
Postmenopausal

Initial STAGE

Initial STAGE

10/02/2025

### Clinical data

#### Breast Imaging :

It Breast GTRASD 5 UOQ suspicious breast lesion

#### Mets Workup :

paus:free  
cap:free

BX  
IOC

ER  
8/8

PR  
8/8

Her 2 Neu  
0

SISH

Ki67  
20

T  
ct4b

N  
0

M  
0

Sonomammography

#### PATIENT HISTORY

### MDT OUTCOME & PLAN

neoadjuvant endocrine followed by left mastectomy

### SIGNATURE

MEDICAL ONCOLOGY

RADIOLOGY

SURGERY

PATHOLOGY

Psychiatric

Nutrition

|                                                |                                                |
|------------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذة / مديحة محمد عبد الغنى | Request No. : 13007679                         |
| Patient No. : 25102080100304                   | Request Date : 22/12/2024                      |
| Path No. : 5137/24                             | Verification Date : 05/01/2025                 |
| Sex : Female                                   | Financial Category : أورام دار السلام - مبالغه |
| Age : 73 Y                                     | Department : Out Patient                       |
| Referring Doc :                                | Registered by : Mrs.Mayada Hegazy              |

### Pathology Report

#### Clinical Data

Left breast mass

#### Nature of specimen

Tru cut biopsy, collected at 22/12/2024 10:30 am

#### Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

#### Microscopic examination

- \* Histological type: Invasive duct carcinoma
- \* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- \* DCIS: No
- \* Angioinvasion: Couldn't be detected in the received biopsy
- \* Perineural invasion: No
- \* IPT ( Positive control showed appropriate reactivity ) :
- \* ER:Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
- \* PR: Strong nuclear reaction in 70 % tumor cells (Allred score 8/8)
- \* HER2: No membrane reaction in of tumor cells (score 0)
- \* Ki 67: 20 % nuclear reaction of tumor cells

#### Diagnosis

Left breast mass, Tru cut biopsy:

Verified by :

Dr.Heba Hasan

Printing Date : 13/01/2025 10:38:51

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



قسم الباثولوجي - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / منيرة محمد عبد الغنى

Patient No. : 25102080100304

Path No. : 5137/24

Sex : Female

Age : 73 Y

Referring Doc :

Request No. : 13007679

Request Date : 22/12/2024

Verification Date : 05/01/2025

Financial Category : اورام ناز السلام - ميثلد

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

### Pathology Report

Invasive duct carcinoma, Grade II

IPT:

\* ER: Positive (score 8/8).

\* PR: Positive (score 8/8).

\* HER2: Negative (score 0).

\* Ki67: Positive in 20% of tumor cells.

Signature

Dr. Rana Fathy

Dr.Heba Hassan

Verified by :

Dr.Heba Hasan

Printing Date : 13/01/2025 10:38:51

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



شرفة رقم (4) - الدور الأول - مبنى رقم (907) - موزنيش الثاني - مصر الجديدة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name: Madeha Mohamed  
Date: 27 January 2025

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**

**Liver: -**

- Average size ( 15.5 cm at the right MCL), displaying bright homogenous echopattern .
- No definite focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

**Gall Bladder: -**

- Average distention with no evidence of stones or mud inside.
- The wall is not thickened.
- CBD is not dilated.

**Spleen:**

Average size with homogenous echopattern, no focal lesions.

**Kidneys: -**

are normal position with average size and shape. reduced parenchymal thickness with moderate increased echogenicity. The cortico-medullary differentiation is not preserved. No stones or back pressure changes .

- Para-aortic and pancreatic area: concealed by gases
- No ascites
- **Urinary Bladder: -**
  - Average filling with normal wall thickness with no stones or focal lesions.
  - Pelvic organs :are unremarkable

**Opinion:**

- Bright liver .
- Bilateral kidney diseases grade III nephropathy .
- Marked colonic gaseous distension .

Best regards  
Dr.Eman Youssef



|                                                    |                   |
|----------------------------------------------------|-------------------|
| Patient Name: Madiha Mohamed Abd Al Ghany Albkraoi | Date : 23/01/2025 |
| Exam : C.T CHEST                                   | Age : 74 Year(s)  |
| Medical ID: 166/2025                               | Pat. ID: 97151    |

**NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

**A reported case of left breast cancer, showing**

**1. Chest:**

- Bilateral breasts are partially out of view, however there is left breast UQ few closely related isodense mass lesions measuring together about 3.6 X 2.1 cm, surrounded by diffuse breast edema and mild increased skin thickening.
- Prominent bilateral axillary lymph nodes much more marked at the left side.
- Few bilateral scattered mostly subpleural tiny pulmonary nodules ranging about 3 to 5 mm.
- Bilateral diffuse ill-defined reticular ground glass opacities with multiple reticulations and fibrotic bands? Nature.
- Few subcentemetric mediastinal lymph nodes, the largest seen at right paratracheal measures about 7x6 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi are seen.

**2. Abdomen and pelvis:**

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Few tiny gall bladder hyperdense stones.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**3. The scanned skeleton:**

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions. No vertebral structural collapse or extra-osseous soft tissue component.
- L4 and L5 mild forward slippage is seen.



|                                                    |                   |
|----------------------------------------------------|-------------------|
| Patient Name: Madiha Mohamed Abd Al Ghany Albkraoi | Date : 23/01/2025 |
| Exam : C.T CHEST                                   | Age : 74 Year(s)  |
| Medical ID: 166/2025                               | Pat. ID: 97151    |

**OPINION:**

- *Left breast mass with diffuse edema and mild skin thickening.*
- *Bilateral scattered pulmonary nodules for follow up.*
- *Bilateral ill-defined ground glass opacities and multiple reticulations and fibrotic bands for clinical correlation and follow up.*
- *No definite hepatic or osseous metastatic deposits on non-contrast basis.*

*Dr.Maie khattab, MSc.*

*Dr. Walid Mohamed, M.D.*



### Women's Health Outreach Program

Name: Madiha Mohamed AbdelGhany

Date: 18-12-2024

### Bilateral full field Digital Mammography

Clinical summary: 73 yrs old female complaining of left breast lump

#### Technique

Medio-lateral oblique and CC views & High resolution ultrasound

#### Detailed Findings

##### Right breast

- Scattered fibroglandular densities ( ACR B )
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and areolar complex.
- Non-specific axillary LNs.

##### Left breast

- Scattered fibroglandular densities ( ACR B )
- Diffuse increase skin thickness and edema pattern associated with LT UOQ area of spiculated distortion
- Ultrasound
- Interstitial Edema pattern and diffuse increase skin thickness associated with LT UOQ hypoechoic spiculated suspicious solid mass lesion seen 2 o'clock zone C measures 2.5x2.3 cm about 9 mm from the skin another similar yet smaller one is seen nearby showing irregular hypoechoic lesion is seen measures 1.3x1 cm with the distance between about 0.6 cm
- No pathological enlarged axillary lymph nodes

#### Impression:

Left breast (BI-RADS 5) : UOQ suspicious breast lesions as described with

Right breast (BIRADS 1): Normal study.

#### Recommendations:

Left breast: for pathological correlation

Best regards,

Dr/ Nour Elhuda Hassan